Walter Reed National Military Medical Center

WRNMMC LOGICOLE EQUIPMENT REQUEST FORM

Required for Medical, Dental, Non-Medical, IT, Furniture, and Durable Equipment Requests.

1. MEDICAL OR DENTAL FACILITY

a. Shipping Address:	b. Priority	c. DMLSS Cust ID	d. LogiCole REF#
Walter Reed National Military Medical Center **Bldg 54 Medical Warehouse** **8901 Wisconsin Ave** **Bethesda, MD 20889-5600**	O-13 Routine	FAHA05	
	e. Department Name: SIMULATION		f. Directorate: DETR

g. Initiator Name	h. Initiator Contact (Electronic)	i. Initiator Phone	j. Date
WYN, MARK J	mark.j.wyn.civ@health.mil	301-400-3839	11/14/2026

2. FUNDING TYPE

◯ OP ◯ O & M ◯ OTHER

3. REQUEST TYPE

◯ New ◯ Trade-In/Replacement ◯ Lease/Cost-per-Test ◯ Software Upgrade ◯ Hardware Upgrade ◯ Retain

4. INTEROPERABILITY

◯ YES ◯ NO

◯ MHS GENESIS ◯ PACS ◯ OTHER

5. ATTACHMENTS

- ☑ Quote(s) *incl. Training & Installation*
- ☑ Maintenance History
- ☑ Manufacturer's Literature, Specifications, Operating Manual
- ☑ Service Manuals (x2)
- ☑ IGCE
- ☑ Market Research
- ☑ Sole Source Justification (J&A) **[REQUIRED when only ONE quote]**
- ☐ Other
  Design Drawings, Turnkey, etc.

REQUIRED FOR REQUEST $100K AND ABOVE

6. EQUIPMENT

a. Standard Nomenclature (Common Name): Patient Simulator, Hi-fidelity

b. Suggested Manufacturer	c. Model Number	
Laerdal Medical	212-05250	
d. Unit Cost: 100000	e. Quantity: 1	f. Total Acquisition Cost: $100,000.00

7. EXISTING EQUIPMENT (FOR TRADE-IN/REPLACEMENT & LEASE/COST PER TEST)

a. Standard Nomenclature (Common Name)		b. Manufacturer	
c. Model Number	d. Serial Number (S/N)	e. Year Purchased	f. DMLSS ECN

g. Additional ECNs & S/Ns. [If Multiple Units] DMLSS Custodian Equipment Detail Sheet is required for each ECN.

j. If Retain, explain. [MANDATORY]

V. FY25

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DETR	SIMULATION	Patient Simulator, Hi-fidelity	

8. ITEM DESCRIPTION / SPECIFICATIONS

a. General design features required to meet existing installation limitations.

(1) Intended Location		(2) Maximum dimensions (inches)		
(a) BLDG	(b) ROOM	(a) Height	(b) Width	(c) Depth
3	4044	71-inches	22-inches	10-inches

(3) Weight not to exceed *(lbs)*	(4) Electrical Voltage available			
	(a) VAC	(b) Hz	(c) Amp	(d) Phase
88lbs	100-240 VAC	50-60 Hz	6.25A	Single

b. General description and characteristics including ALL components and accessories. *(Attach manufacturer's literature and quotation)*

The SimMan Critical Care is a tetherless, full-body patient simulator built for high-fidelity training in respiratory care, anesthesia, critical care, and emergency medicine. It integrates onboard ASL 5000 lung technology so learners can connect any real mechanical ventilator in any mode and move the patient seamlessly from pre-hospital care to the ICU. Each SimMan Critical Care system comes complete with the full-body adult manikin—configured with either articulating NIBP arms or RFID medication-

c. Essential Characteristics. Description must be generic, not manufacturer specific. *(Detailed, nontechnical, functional description, including accessories and options, of the minimum features and capabilities required to enable completion of intended task. Do not use manufacturer specific terms, model numbers, catalog numbers, and proprietary information)*

simulate complications such as tongue swelling, laryngospasm, pharyngeal swelling, and jaw clenching (trismus). For cardiovascular and resuscitation training, the system must output 12-lead ECG signals, support 4-wire cardiac monitoring, pacing, cardioversion, and live defibrillation (via direct pads or training studs), generate palpable pulses (carotid, femoral, radial, brachial, and pedal) that automatically adjust to simulated blood pressure, and provide realistic chest resistance that tracks compression depth, release, and rate in real time according to standard resuscitation guidelines. Additionally, the manikin must simulate

(5) Mounting requirements. *(i.e., Seismic, fastened to deck, overhead, bulkhead, etc.)*

The system requires no permanent structural mounting, seismic anchoring, bulkhead fastening, or overhead suspension.

(6) Utility Requirements

☐ Water ☐ Drain ☐ Heat Dissipation ☐ Temperature Regulation ☐ Gases

(7) Other unique requirements, not previously mentioned. *(i.e., surge protection, security requirements such as locks, cabinets, doors, etc.)*

The system requires no dedicated facility utility hookups—such as hardwired electrical circuits, piped medical gases, compressed air lines, or plumbing/drainage connections. All utility needs are met through standard room infrastructure and self-contained internal systems

(8) Is the ability of the manufacturer to provide local maintenance and support critical? ◯ Yes ◯ No

(9) If yes, describe the support required, acceptable response time, and any factors an offeror should be made aware of. *(i.e., limited access to base, citizenship requirements, etc.)*

because the system depends on wireless communication and scenario-control software to interface with bedside monitors and debriefing equipment, on-site field service engineers and technical specialists are essential for resolving network configuration conflicts, executing firmware updates, performing annual calibrations, and preserving original manufacturer warranty coverage

(10) Is operator training required? ◯ Yes ◯ No

If Yes, explain.

misuse of supplemental oxygen and live defibrillation equipment can cause severe internal damage to pneumatic valves and electronic circuitry or create safety hazards for staff and learners. Structured initial training ensures simulation personnel can execute pre-scenario system checks, manage consumable replacements, perform routine post-use purging and cleaning, and

V. FY25

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9. JUSTIFICATION

(Use additional sheet(s) to answer. Indicate Medical/Dental Facility name and LogiCole Ref # on each page. Ensure to provide all information requested)

a. How is the function of the item currently being accomplished?

reached the end of its serviceable lifecycle, experiencing degraded internal electro-mechanical components, intermittent telemetry failures, and declining reliability that disrupt scheduled training exercises. While the center maintains other patient simulators in its inventory, those assets cannot absorb this workload because they lack the specialized high-fidelity lung mechanics required to interface with real clinical mechanical ventilators, or they are fully committed to other concurrent training programs under existing fleet rotation schedules. Consequently, this acquisition is a direct lifecycle replacement required to retire a no-longer-functional unit and restore uninterrupted, high-acuity clinical simulation capacity without displacing other active

b. Provide information on current staff, by specialty, that is available to use the equipment and what, if any, additional staffing will be required.

The Department of Simulation currently operates with 21 personnel across clinical, technical, operational, and administrative specialties. Of that workforce, 14 staff members directly operate, program, maintain, or facilitate clinical instruction using the high-fidelity critical care simulator, while the remaining 7 personnel provide essential scheduling, logistics, standardized patient, and administrative support.

c. Will requested item be used in conjunction with other equipment within the facility? *(existing or proposed)* ◯ Yes ◯ No

If Yes, explain.

Yes, the patient simulator is designed to operate fully integrated with a wide range of existing medical, clinical, and simulation equipment within the facility. In daily operations, it will interface directly with real clinical mechanical ventilators across volume, pressure, and spontaneous breathing modes to simulate complex pulmonary mechanics, positive end-expiratory pressure (PEEP) management, and ventilation weaning scenarios. It will also connect with dedicated simulated bedside monitors and facility

d. Provide information on similar equipment that is currently available at the facility and the usage of that equipment *(existing or proposed)* even if it is in another department.

10. DEPARTMENT POC

(1) DEPARTMENT PROPERTY CUSTODIAN

a. Name	b. Signature	
Mark J. Wyn		
c. Contact (Electronic)	d. Telephone	e. Date
mark.j.wyn.civ@health.mil	301-400-3839	11/14/2026

(2) DEPARTMENT HEAD / SERVICE CHIEF

a. Name	b. Signature	
Sarah J. Krajnik		
c. Contact (Electronic)	d. Telephone	e. Date
sarah.j.krajnik.civ@health.mil	301-400-0501	11/14/2026

V. FY26